

# CPTI Post Site Survey

Thank you for your continued partnership in the Emergency Departments LEADING the transformation of Alzheimer's and Dementia care (ED-LEAD) study. The CPTI portion is now complete, and as next steps we wanted to garner feedback on implementation and sustainability. This survey should take about 15-20 minutes. Thank you in advance for your participation.

Please answer the following questions to the best of your ability.

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Select the health system name

- ☐ Advocate Aurora Health
- ☐ Atrium Health Wake Forest
- ☐ Brigham & Women
- ☐ Henry Ford Health System
- ☐ Harvard Mass General
- ☐ MAYO Clinic
- ☐ Mount Sinai
- ☐ Northwell
- ☐ Northwestern
- ☐ NYU
- ☐ Ochsner
- ☐ Pittsburgh
- ☐ Rochester
- ☐ Colorado
- ☐ UW Madison

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[survey-url]

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Select the site name

- ☐ Austin
- ☐ Baptist-Atrium
- ☐ Baton Rouge
- ☐ BayCare
- ☐ Brigham & Womens Faulkner
- ☐ Cabarrus
- ☐ East
- ☐ East Madison
- ☐ Highlands Ranch
- ☐ Huntington
- ☐ Huntley
- ☐ Jackson
- ☐ Lake Forest
- ☐ LIJ Hospital
- ☐ LIJ Valley Stream
- ☐ Long Island Community
- ☐ McHenry
- ☐ McKeesport
- ☐ Morningside
- ☐ Mount Sinai Hospital
- ☐ New Orleans
- ☐ North Shore
- ☐ Northwestern Memorial
- ☐ Noyes
- ☐ Pineville
- ☐ Poudre Valley
- ☐ Rockies
- ☐ Shadyside
- ☐ Sinai
- ☐ St Lukes Medical Center
- ☐ St Lukes South Shore
- ☐ Staten Island South
- ☐ Syosset
- ☐ Tisch
- ☐ Union
- ☐ University City
- ☐ UPMC Presbyterian
- ☐ West
- ☐ West Bank
- ☐ West Bloomfield

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Site Randomization

- ☐ CPTI only
- ☐ ECR/CPTI
- ☐ CPTI/NLTC
- ☐ All three

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Full name of person(s) completing this survey,

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### CPTI CDS Build

Below are screenshots we have on file from the beginning of the intervention.

1. Disposition Alert

2. CPTI information on the After Visit Summary

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Below are screenshots we have on file from the beginning of the intervention.

1. Disposition Alert
  
2. CPTI information on the After Visit Summary

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Below are screenshots we have on file from the beginning of the intervention.

1. Disposition Banner Alert
  
2. CPTI information on the After Visit Summary

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Below are screenshots we have on file from the beginning of the intervention.

1. Disposition Alert
  
2. CPTI information on the After Visit Summary

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Below are screenshots we have on file from the beginning of the intervention.

1. Disposition Alert
  
2. CPTI information on the After Visit Summary

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Below are screenshots we have on file from the beginning of the intervention.

1. Disposition Alert
  
2. CPTI information on the After Visit Summary

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Below are screenshots we have on file from the beginning of the intervention.

1. Disposition Alert
- We do not have a screenshot on file. Please click disposition alert below and upload.
2. CPTI information on the After Visit Summary

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Review the screenshots above. Did either of these components of the build change during implementation?

- ☐ Disposition Alert  
☐ CPTI Language on the AVS

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Please upload a screenshot of the updated disposition alert.

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Please upload a screenshot of the updated CPTI AVS language.

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We have the below service area codes/description of the geographic area covered by your EMS agency for CPTI.

- ☐ Service area changed  
☐ Service area stated the same

Charlotte metro region, Charlotte-Mecklenburg County, Forsyth County, Winston Salem, High Point, Davie County

At the end of the intervention period, was this still accurate or did it change during implementation?

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We have the below service area codes/description of the geographic area covered by your EMS agency for CPTI.

- ☐ Service area changed  
☐ Service area stated the same

Manhattan

At the end of the intervention period, was this still accurate or did it change during implementation?

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We have the below service area codes/description of the geographic area covered by your EMS agency for CPTI.

- ☐ Service area changed  
☐ Service area stated the same

They serve a 45-minute catchment area around the hospital at a minimum, but also as far as their 911 response area. We provide 911 ambulance service to several small mountain communities in Larimer County and rural agricultural communities in Weld County. While the number of patients from these locations would likely be limited, being able to integrate our Community Paramedic program with our 911 Paramedic coverage would be meaningful.

At the end of the intervention period, was this still accurate or did it change during implementation?

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We have the below service area codes/description of the geographic area covered by your EMS agency for CPTI.

- ☐ Service area changed  
☐ Service area stated the same

Jackson County

At the end of the intervention period, was this still accurate or did it change during implementation?

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We have the below service area codes/description of the geographic area covered by your EMS agency for CPTI.

- ☐ Service area changed  
☐ Service area stated the same

Mower County

At the end of the intervention period, was this still accurate or did it change during implementation?

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We have the below service area codes/description of the geographic area covered by your EMS agency for CPTI.

- ☐ Service area changed  
☐ Service area stated the same

50 mile radius from Huntley excluding areas outside of Illinois; we are using zip codes to confirm inclusion/exclusion

At the end of the intervention period, was this still accurate or did it change during implementation?

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We have the below service area codes/description of the geographic area covered by your EMS agency for CPTI.

- ☐ Service area changed  
☐ Service area stated the same

10025, 10128, 10024, 10021, 10044, 1023, 10019, 10022, 10036, 10017, 10018, 10016, 10001, 10011, 10010, 10014, 10009, 10003, 10002, 10012, 10013, 10007, 10038, 10280, 10006, 10005, and 10004.

At the end of the intervention period, was this still accurate or did it change during implementation?

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Briefly describe the change in service area coverage.

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Was there a hardstop associated with documentation of care partner information (e.g., the discharge paperwork could not be printed until care partner information was documented)?

- ☐ Yes  
☐ No  
☐ Other

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Please describe

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Was the CPTI flyer included in the discharge paperwork?

- ☐ Yes  
☐ No  
☐ Other

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Please describe

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**EMS Agency Background/Context**

Describe how the EMS agency was selected for this site.

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What is the ownership type of the EMS agency you partnered with?

- ☐ Hospital
- ☐ Municipal (Third Service EMS)
- ☐ Fire
- ☐ Non-Profit
- ☐ Private/commercial
- ☐ Other, please describe

Briefly describe the ownership type of the EMS agency you partnered with.

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Before implementing the CPTI, did this EMS agency already have a community paramedicine program?

- ☐ Yes
- ☐ No

Briefly describe the community paramedicine program before CPTI started (e.g., what funded programs were in place, who did these programs serve, etc.)

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Describe how the coaches were selected for the CPTI.

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How would you rate EMS leadership support of the CPTI program (e.g., did they "buy-in" to the program)?

- ☐ Very high
- ☐ High
- ☐ Neutral
- ☐ Low
- ☐ Very low

Did the coaches have access to the site's EHR platform during CPTI implementation?

- ☐ Yes, all coaches has access
- ☐ Yes, some coaches has access
- ☐ No

Please describe any ways that access (or not having access) to the EHR influenced the coaches' work.

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**Integration of CPTI in the ED**

How difficult was it to integrate CPTI into existing in-ED workflows?

- ☐ Not at all difficult
- ☐ Slightly difficult
- ☐ Moderately difficult
- ☐ Very difficult
- ☐ Extremely difficult

Please elaborate on what factors influenced the difficulty of integrating CPTI into existing workflows.

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How did your site socialize CPTI information to ED staff? Please check all that apply.

- ☐ Email to staff
- ☐ Announcements at staff huddles/meetings
- ☐ Inclusion in staff/department newsletters
- ☐ Signage in the ED
- ☐ Paramedic coach(es) visited the ED
- ☐ Other, please describe

Briefly describe how your site socialized CPTI information to the ED staff.

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How often do you think eligible patients were informed about and encouraged to participate in the CPTI program during their ED visit?

- ☐ Always  
☐ Often  
☐ Sometimes  
☐ Rarely  
☐ Never  
☐ Not Sure

Describe what factors at your site influenced how often eligible patients were informed about CPTI during ED visits?

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Did your site implement any of the following in-ED strategies that influenced which patients were referred to the CPTI? Select all that apply

- ☐ None - all eligible patients were referred to the CPTI, potentially pending Program Coordinator screening/review after discharge  
☐ Geographic range programmed in the EHR (CPTI service area)  
☐ Provider order required in the ED EHR  
☐ Clinical staff required to confirm that CPTI was discussed with the patient / care  
☐ Clinical staff documentation in the EHR required if a patient / care partner declined  
☐ Other, please describe

Briefly describe other in-ED strategies that influenced which patients were referred to the CPTI.

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### CPTI Referral & Enrollment Processes

Describe your site's experience with the process of referring eligible patients to the CPTI (e.g., after the patient was discharged from the ED). What challenges did you face in the referral process? What worked well at your site?

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Describe your site's experience with enrolling patients in the CPTI (e.g., getting them to agree to the home visit). What challenges did you face enrolling dyads? What worked well at your site?

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How adequate was program coordinator coverage to meet CPTI duties, given the patient volume at your site?

- ☐ Coverage was far below what was needed  
☐ Coverage was somewhat below what was needed  
☐ Coverage met needs about half the time  
☐ Coverage generally met needs  
☐ Coverage fully met needs most of the time

### EMS CPTI Implementation

How adequate was coach coverage to meet CPTI duties, given the patient volume at your site?

- ☐ Coverage was far below what was needed  
☐ Coverage was somewhat below what was needed  
☐ Coverage met needs about half the time  
☐ Coverage generally met needs  
☐ Coverage fully met needs most of the time

How difficult was it to integrate CPTI into existing workflows at the EMS agency?

- ☐ Not at all difficult  
☐ Slightly difficult  
☐ Moderately difficult  
☐ Very difficult  
☐ Extremely difficult

Please elaborate on what factors influenced the difficulty of integrating CPTI into existing EMS workflows.

Describe your site's experience with implementing the CPTI (e.g., coaches completing the home visit and follow-up calls). What challenges did you face related to implementing the CPTI? What worked well at your site?

How confident are you that CPTI was being delivered as intended?

- ☐ Extremely confident  
☐ Very confident  
☐ Moderately confident  
☐ Slightly confident  
☐ Not at all confident  
☐ I don't know

Please elaborate on what influenced your level of confidence in CPTI being implemented as intended at your site.

Were there any unintended consequences (positive or negative) associated with the implementation of the CPTI at the EMS agency?

- ☐ Yes  
☐ No

Please describe the unintended consequences associated with the implementation of the CPTI at the EMS agency.

Do you think this program \_\_\_\_\_ health inequities in your institution?

- ☐ Improved  
☐ Worsened  
☐ Did not impact

### Ongoing Quality Improvement & Coordination

Please describe how CPTI data were reviewed and monitored at your site (e.g., who was involved, how often reviews occurred, and what data were typically reviewed).

Did the EMS agency actively address implementation barriers when identified (e.g. coach performance concerns, missing care partner information, etc.)?

- ☐ Yes, proactively  
☐ Yes, when prompted  
☐ Rarely  
☐ No

Briefly comment on your selection above.

How effective was coordination with the EMS agency in supporting CPTI implementation?

- ☐ Very effective  
☐ Somewhat effective  
☐ Neither effective nor ineffective  
☐ Somewhat ineffective  
☐ Very ineffective

Briefly comment on how effective the coordination with the EMS agency was in supporting CPTI implementation.

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### Sustainability & Future Directions

Will you be sustaining CPTI beyond the study period?

- ☐ Yes  
☐ No

What changes or adaptations would be needed to sustain CPTI at your site long-term?

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### Follow-up

Would you be interested in participating in a qualitative interview to discuss barriers and facilitators that impacted implementation of CPTI at your site?

- ☐ Yes  
☐ No

### Name/Demographic Information

Title

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Select your role. Check all that apply.

- ☐ Practicing Emergency Medicine Clinician  
☐ Site PI  
☐ Researcher  
☐ Provider Champion  
☐ Nurse Champion  
☐ Coordinator

Age

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How many years have you been practicing as a physician or nurse? (for Physicians excluding residency)

- ☐ 0-14  
☐ 15-25  
☐ 25+

### ED Variables

What is your hospital type?

- ☐ Academic  
☐ Community  
☐ Nonprofit  
☐ For profit  
☐ Governmental

What US region is your hospital located in?

- ☐ Northeast  
☐ Midwest  
☐ Southeast  
☐ Southwest  
☐ West

Is your hospital in a rural area?

- ☐ Yes  
☐ No

Does your hospital have a geriatric certification?

- ☐ Yes  
☐ No

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EM Residency training site

☐ Yes  
☐ No

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Trauma Center

☐ Yes  
☐ No

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Dedicated EM social workers/care manager

☐ Yes  
☐ No

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What is your hospital system ID number?

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